Nintedanib · continuation

To continue controlling lung cancer after combination treatment, while benefit continues and side effects remain acceptable.

My overall treatment

Take the prescribed nintedanib dose twice daily every day. Reviews are organised in 28-day cycles; there is no planned tablet-free week. This phase starts only after at least 4 combination cycles and team approval.

Continuous oral phase

Nintedanib twice daily every day;28-day review cycles. No planned tablet-free week.

A typical 28-day cycle

Day 1–28
Nintedanib every day

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Phase and recovery review** Required by regimen	At least 4 completed combination cycles and consultant approval; confirm new capsule plan
During	**Blood count, kidney and liver tests** Required by regimen	Before each 28-day cycle; liver tests at least monthly, more often if needed
During	**Blood pressure, symptoms and medicines** Required by regimen	BP periodically/as indicated; extra tests for liver-risk factors or symptoms
After / review	**Response, scans and recovery** Team-agreed plan	Agree response and follow-up timing; no fixed scan or tumour-marker interval

Between treatments

- Diarrhoea, poor intake or vomiting needs early advice; follow your written fluid and medicine plan.
- Keep monthly liver tests even when feeling well; report new symptoms promptly.

Bring your medicine list and report new symptoms before dosing. Confirm test appointments, the current phase and daytime / out-of-hours contact plan.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1. Head / neurological symptoms

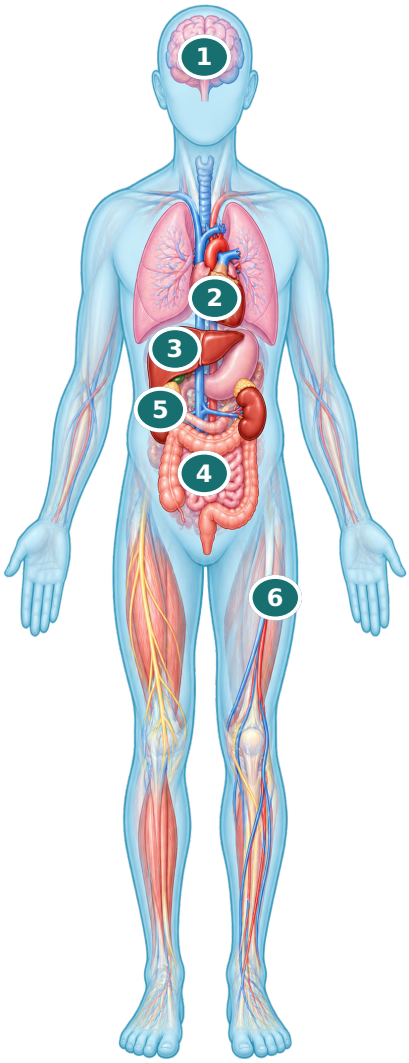
Nintedanib

New severe headache, confusion, seizure or visual change needs urgent advice.

2. Blood vessels / pressure

Nintedanib

Chest pain, sudden breathlessness or a swollen painful leg needs urgent assessment.



3. Liver

Nintedanib

Attend monthly liver tests; report yellow skin/eyes or dark urine.

4. Bowels / sickness

Nintedanib

Call early for diarrhoea or vomiting; severe abdominal pain is urgent.

5. Kidneys / urine

Nintedanib

Report swelling, foamy urine or much less urine.

6. Bleeding / wounds

Nintedanib

Report bleeding or poor wound healing. Tell oncology before procedures.

Selected effects, not every possible effect or a prediction. Match each number to its symptom box. Blood, skin and nerve markers are schematic.

Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment.

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Keep your treatment details to hand

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Nintedanib continuation

Nintedanib twice daily every day;28-day review cycle. No planned break.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — not a fixed treatment course.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

Hospital / team: _____ Backup contact route: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.